procarbazine, lomustine and vincristine (PCV) chemotherapy · 56-day cycle

PCV · 56 days NCCP 00658: adjuvant treatment of grade II glioma after radiotherapy. Use the plan matched to your diagnosis and phase; do not borrow another glioma schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1: lomustine

Active medicines: Lomustine

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/dose	Before Day 1 lomustine, then the prescribed delayed-emesis cover National Cancer Control Programme (NCCP) V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.

Additional phase-specific support

- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00658 v2 · p4 · NCCP oral-SACT antiemetics V6: Table 5 p8; timing p4; interactions p11

NCCP regimen-specific support · Sources checked 2026-11-05 · SACTCheck v0.84.38 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Days 8-21: procarbazine; vincristine on Day 8

Active medicines: Procarbazine + Vincristine on Day 8

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/dose	Daily during procarbazine Days 8-21; confirm the prescribed stop date after this multiday phase National Cancer Control Programme (NCCP) V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.
Constipation prevention	Complete the locally prescribed laxative and dose	With vincristine exposure; review bowel function before each infusion NCCP procarbazine, lomustine and vincristine (PCV) supportive care recommends prophylaxis. Severe pain, vomiting or inability to pass stool/wind requires urgent assessment, not automatic extra laxatives.

Additional phase-specific support

- NCCP treatment table specifies procarbazine Days 8-21; the supportive-care paragraph says Days 8-22. Do not add chemotherapy on Day 22. Pharmacy confirms the antiemetic stop date.

- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00658 v2 · p4 · NCCP oral-SACT antiemetics V6: Table 5 p8; timing p4; interactions p11

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Day 29: vincristine only

Active medicines: Vincristine

Emetogenic risk: Minimal

Medicine	Dose / route	When
Constipation prevention	Complete the locally prescribed laxative and dose	With vincristine exposure; review bowel function before each infusion National Cancer Control Programme (NCCP) procarbazine, lomustine and vincristine (PCV) supportive care recommends prophylaxis. Severe pain, vomiting or inability to pass stool/wind requires urgent assessment, not automatic extra laxatives.

Additional phase-specific support

- No routine antiemetic prophylaxis is required for vincristine alone; prior symptoms or other medicines may require an individual rescue plan.
- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last systemic anticancer therapy (SACT) day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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